Systemic Fracture: A Comprehensive Analysis of the Structural Failure, Identity Crisis, and Militarization of United States Emergency Medical Services

Executive Summary

The United States Emergency Medical Services (EMS) system currently exists in a state of profound structural dissonance, a condition exacerbated by sixty years of misclassification, bureaucratic neglect, and internecine labor conflicts. Unlike its public safety counterparts—law enforcement and fire suppression—EMS possesses no unified federal home, no guaranteed tax base in many jurisdictions, and no distinct professional identity recognized uniformly by the Bureau of Labor Statistics. It is a profession stranded between the clinical demands of healthcare and the logistical framework of transportation.

This report serves as a "Deep Dive" into the systemic failures that have brought US EMS to the brink of collapse in 2025. It traces the lineage of dysfunction from the "Original Sin" of the 1966 *Accidental Death and Disability* White Paper, which tethered EMS to the Department of Transportation (DOT), to the modern dissolution of high-performance Public Utility Models (PUM) like Fort Worth's MedStar in favor of fire-based hegemony.

Furthermore, this analysis interrogates the active legislative battles of the 119th Congress—specifically H.R. 3791, H.R. 3443, and H.R. 2220—which seek to rectify decades of data suppression and reimbursement misalignment. These bureaucratic struggles are juxtaposed against a rapidly militarizing domestic landscape: the 2025 executive designation of Fentanyl as a Weapon of Mass Destruction (WMD), the renaming of the Department of Defense to the "Department of War," and the imposition of draconian digital surveillance laws in states like Wisconsin and Michigan that threaten the encrypted communications infrastructure vital to modern telemedicine.

The findings suggest that without a radical federal reclassification of EMS as an essential healthcare service—decoupled from transportation metrics and shielded from the labor monopolization of fire unions—the system faces a catastrophic "exodus" of clinical talent to the nursing sector and a collapse of prehospital care capacity in rural and urban centers alike.

Section I: The Anatomy of the Original Sin — 1966 and

the Department of Transportation

The structural incoherence of modern American EMS is not an accident of evolution but the direct result of a specific policy decision made in the mid-20th century. To understand why a paramedic in 2025 cannot bill for treating a diabetic patient on-scene without transport, one must return to the geopolitical and industrial context of 1966.

1.1 The Neglected Disease of Modern Society

In 1966, the National Academy of Sciences/National Research Council (NAS/NRC) published the seminal white paper, *Accidental Death and Disability: The Neglected Disease of Modern Society*.[1] This document is widely revered as the birth certificate of modern EMS, yet it contains the genetic code for the system's current dysfunction.

The authors of the White Paper were motivated by a stark statistical reality: in 1965, 52 million accidental injuries killed 107,000 Americans, causing a societal cost of approximately $18 billion (roughly $129.4 billion in adjusted dollars).[3] Motor vehicle crashes accounted for nearly half of those deaths. The disparity between military and civilian survival rates was glaring; a soldier wounded in the jungles of Vietnam had a higher statistical probability of survival than a motorist injured on a California freeway.[2] The military possessed a vertically integrated system of rapid evacuation, field stabilization, and definitive care; the civilian world relied on a patchwork of funeral home hearses, volunteer rescue squads, and law enforcement wagons with no medical equipment.[2]

The White Paper identified five pillars for trauma reduction, modeled on the Haddon Matrix: safer people, safer roads, safer vehicles, safer speeds, and post-crash care.[1] It was this final pillar—post-crash care—that necessitated the creation of a formal EMS system. However, the framing of the problem largely determined the solution. Because the primary vector of trauma was the automobile, the legislative response—the National Highway Safety Act of 1966—placed the nascent EMS apparatus under the jurisdiction of the Department of Transportation (DOT), specifically the National Highway Traffic Safety Administration (NHTSA).[1]

1.2 The "Cargo" Paradigm vs. Clinical Care

The placement of EMS within the DOT established a "transportation-centric" identity that persists to this day. Under the DOT model, the ambulance is viewed legally and financially as a vehicle of transport—a specialized form of shipping—rather than a mobile clinical unit.

The Reimbursement Trap:

This classification rippled into the reimbursement structures established by the nascent Medicare and Medicaid programs. Because EMS was housed under DOT, the Centers for Medicare & Medicaid Services (CMS) codified payment based on the transport of a patient to a hospital. The "service" rendered was defined as the movement of the patient, not the medical assessment or intervention provided.1

- **The "You Call, We Haul" Incentive:** This funding mechanism created a perverse incentive known as the "load and go" model. It financially discouraged on-scene

stabilization, treat-and-release protocols, or transport to alternative destinations (such as urgent care or mental health facilities). If the wheels of the ambulance did not turn, no revenue was generated.[5]

- **Cost of Readiness Ignored:** Unlike fire departments or police forces, which are funded to be *ready* regardless of call volume, EMS reimbursement only covered the active transport phase. This left the "cost of readiness"—the expense of having a fully staffed, equipped ICU on wheels waiting for a call—unfunded by federal insurance programs.[5]

This misalignment has created a catastrophic bottleneck in modern healthcare. As noted in current analyses, the inability to bill for non-transport services forces EMS agencies to transport non-emergent patients to already overcrowded Emergency Departments (EDs). This contributes to "wall time"—the period where an ambulance crew is stuck at the hospital waiting to offload a patient—which strips communities of ambulance coverage and increases burnout among crews.[5]

1.3 The Lost Opportunity of HEW/HHS

The structural flaw was identified almost immediately. In 1972, a follow-up NAS/NRC report, *Roles and Resources of Federal Agencies in Support of Comprehensive Emergency Medical Services*, explicitly criticized the federal fragmentation. It urged the integration of all EMS efforts into the Department of Health, Education, and Welfare (DHEW), the precursor to the Department of Health and Human Services (HHS).[7]

The report argued that EMS was fundamentally a medical practice, not a highway safety measure. While the EMS Systems Act of 1973 briefly created a categorical grant program within DHEW to foster regional systems, this was dismantled by the Omnibus Budget Reconciliation Act of 1981, which converted EMS funding into block grants.[7] This devolution allowed states to deprioritize EMS, cementing the DOT as the only federal agency with a consistent, albeit limited, mandate (highway safety standards) for EMS.

Consequently, EMS lacks a Cabinet-level advocate for its clinical mission. While police have the Department of Justice and fire services have the U.S. Fire Administration (under FEMA/DHS), EMS remains a stepchild of the highway administration, fighting for scraps of funding tied to asphalt rather than patient outcomes.[4]

Section II: The "Neither/Nor" Identity Crisis — Fire vs. Third Service vs. Private

The regulatory vacuum left by the federal government allowed three distinct models of EMS delivery to emerge, creating a fractured labor market and intense inter-agency rivalry. The primary conflict today exists between the Fire-based EMS model (supported by the International Association of Fire Fighters - IAFF) and the "Third Service" or Public Utility Model (PUM).

2.1 The Rise of Fire-Based EMS Hegemony

As structural fires in the United States declined precipitously due to improved building codes and fire prevention materials, municipal fire departments faced a crisis of relevance and budget justification.[9] To preserve staffing levels and funding, fire departments aggressively expanded into EMS.

- **The "Dual Role" Model:** In this system, personnel are cross-trained as firefighter/paramedics. The argument is one of efficiency: a standing army of firefighters can be utilized for medical calls.[10]
- **The "Cash Cow" Critique:** Critics, including independent EMS advocates and labor analysts, argue that EMS has become the "cash cow" for fire departments, justifying multi-million dollar budgets and heavy apparatus for calls that are 90% medical.[9]
- **The FLSA "80/20" Battle:** A critical, often overlooked aspect of this history involves the Fair Labor Standards Act (FLSA). Firefighters enjoy a partial exemption from overtime rules (Section 7(k)), allowing for longer shifts without immediate overtime pay. In the 1990s, legal battles (e.g., *West v. Anne Arundel County*, *Alex v. City of Chicago*) challenged whether cross-trained paramedics doing mostly medical work qualified for this exemption. The Department of Labor eventually ruled that as long as they are trained in fire suppression and have the legal authority to fight fires, they qualify, regardless of how much time is spent on EMS.[11] This solidified the financial viability of the fire-based model by keeping labor costs artificially lower than if they were subject to standard 40-hour workweeks.

2.2 Case Study: The Fall of MedStar and the Fire Takeover

The most significant recent battle in this structural war occurred in Fort Worth, Texas, with the 2024-2025 dissolution of MedStar Mobile Healthcare in favor of a fire-based system. This case serves as a bellwether for the future of the Public Utility Model.

The Rise of MedStar:

MedStar was a classic Public Utility Model (PUM) or "Third Service" agency. It operated independently of the police and fire departments, solely focused on clinical care and transport. For decades, it was regarded as a high-performance system, utilizing data to deploy ambulances dynamically (System Status Management) rather than tethering them to static fire stations.12

The Collapse (2024-2025):

In May 2024, the Fort Worth City Council voted to dissolve MedStar and transfer EMS responsibilities to the Fort Worth Fire Department (FWFD).12

- **Financial Unsustainability:** MedStar operated on a fee-for-service model. With declining reimbursement rates from Medicare/Medicaid and a rise in uninsured patients, the independent model became financially precarious. The city refused to subsidize the independent authority but was willing to inject tax dollars into its own fire department to take over the service.[12]
- **Union Strategy (Local 440):** The transition was heavily championed by IAFF Local 440. The union argued that integrating EMS into the fire department would provide civil service protection and better benefits for paramedics, who would transition from

private/quasi-public employment to sworn city positions.[14] The union utilized GIS studies to argue that fire station placement offered better coverage, despite the static nature of stations versus dynamic posting.[14]

- **The "Single Role" Compromise:** Unlike many fire systems that require all staff to fight fires, the Fort Worth model created a "single role" civilian EMS classification within the fire department. However, friction immediately arose regarding dispatchers. The union pushed for dispatchers to be sworn firefighters, arguing they are better equipped for stress, while the city aimed for civilian dispatchers to save costs. This highlights the continuous struggle for union expansion into all facets of emergency response.[16]

Operational Consequences:
The transition, officially completed in July 2025, came with a $10 million price tag and immediate operational degradation.

- **Response Time Lag:** Post-transition data from late 2025 indicates that the fire-based system is hitting its 90% response time target at **11 minutes and 9 seconds**, significantly slower than the 8-minute goal and 30 seconds slower than MedStar's performance the previous year.[17] Fire officials attribute this to "growing pains" and new systems, but it underscores the loss of the PUM's specialized focus on deployment logistics.
- **Cultural Absorption:** Over 600 MedStar employees were absorbed into the fire department. While this preserved jobs, it effectively eliminated an independent, medically-focused voice in the region, subsuming EMS culture into the paramilitary culture of the fire service.[18]

2.3 The Private Sector and "High Performance" Reality

While the fire service consolidates power, the private sector (represented by companies like AMR/GMR) continues to operate largely in the "safety net" or interfacility transport space.

- **The Profit Motive:** Private EMS is driven by contract compliance and profit margins. This often leads to lower wages for providers compared to fire or third-service agencies, fueling high turnover.[20]
- **Agility vs. Stability:** Private companies offer municipalities the ability to outsource risk (liability, billing, staffing) but at the cost of control. When a private provider fails or goes bankrupt, the municipality is left scrambling, often leading to a revert-to-fire scenario.[21]

Table 1: Comparative Analysis of EMS Delivery Models

Feature	Fire-Based EMS	Third Service (Public Utility)	Private Contract
Primary Focus	Fire Suppression & Rescue	Clinical Medicine & Transport	Profit & Contract Compliance
Staffing Model	Dual-Role (FF/Medic) or Single	Single Role (Medic only)	Single Role (Medic only)
Union Representation	High (IAFF)	Moderate (AFSCME/Teamsters)	Low / Fragmented
Funding Source	Tax Base + Billing	Billing +/ Tax Subsidy	Billing (User Fee)

Political Clout	Very High	Low	Low (Lobbying via AAA)
Cost to Municipality	High (Personnel/Pensions)	Moderate	Low (often zero subsidy)
Response Logic	Static (Station-based)	Dynamic (SSM/Posting)	Dynamic (Profit-driven)

Section III: Current Legislative Battles — The Fight for Recognition and Data

The federal neglect of EMS has spawned a flurry of legislative attempts to fix the "Original Sin" through piecemeal bills. These bills, active in the 119th Congress (2025-2026), highlight the desperate need for data parity and reimbursement reform.

3.1 H.R. 3791: The EMS Counts Act

Objective: To require the Secretary of Labor to revise the Standard Occupational Classification (SOC) System to accurately count EMS practitioners.[22]
The Problem: Currently, the Bureau of Labor Statistics (BLS) misclassifies thousands of EMS providers. A firefighter who is cross-trained as a paramedic is often counted solely as a firefighter. This results in a "chronic undercount" of the EMS workforce, masking the severity of the shortage and skewing federal grant allocations.[24]

- **Sponsors:** Rep. Glenn Thompson (R-PA) and Rep. John Mannion (D-NY).[22]
- **The Coalition:** The bill has rare bipartisan support and is endorsed by both NAEMT (National Association of Emergency Medical Technicians) and IAFF. For the IAFF, this bill is strategic: it ensures their members (dual-role providers) are recognized for their medical contributions without stripping them of their firefighter status, which is crucial for their retirement and benefit structures (often more generous than civilian EMS).[24]

3.2 H.R. 3443: Modernizing EMS Delivery Act (The "MEDS" Bill)

Objective: To amend the Social Security Act to create a model for Medicare reimbursement that includes "Treatment in Place" (TIP) and accounts for the cost of readiness.[27]
The Transformation: This bill attacks the DOT legacy of "transport-only" reimbursement. It proposes:

1. **Lump Sum Payments:** Moving away from fee-for-service toward monthly/quarterly payments to maintain readiness. This acknowledges that the value of EMS is availability, not just the ride. It specifically includes costs for maintaining supplies of **blood products** and essential medications, which are often in shortage.[27]
2. **Treatment in Place (TIP):** Allowing reimbursement for care provided on-scene without transport to a hospital. This is critical for reducing ED overcrowding ("wall time") and utilizing community paramedicine to treat chronic conditions or minor injuries at home.[27]

3. **Medical Direction:** It specifically mandates a study on the value of EMS physician oversight. The National Association of EMS Physicians (NAEMSP) strongly supports this, arguing that physician oversight is the only way to ensure quality in a modernized, clinically focused system.[29]

3.3 H.R. 2220: The PARA-EMT Act

Objective: To address the workforce shortage by streamlining the transition of military medics into civilian EMS roles.[30]
Context: With a 20-30% annual turnover rate in civilian EMS, this bill seeks to utilize veterans to plug the gap. It provides grants to states to expedite licensure for veterans with military medical training, bypassing redundant training requirements.[30] This aligns with the 2025 geopolitical context of an increasingly militarized domestic preparedness stance, creating a pipeline from the "Department of War" to civilian streets.

3.4 The Lobbying Ecosystem

The lobbying landscape is a triad of competing interests:

- **IAFF (Fire):** The dominant force. They focus on protecting the fire-based model, securing SAFER grants (Staffing for Adequate Fire and Emergency Response), and ensuring dual-role benefits. They spend significantly on lobbying to ensure EMS remains integrated with fire suppression, viewing independent EMS as a threat to their staffing models.[32]
- **NAEMT (Practitioners):** Represents the clinical workforce. Their primary push is for the "EMS Counts Act" and workforce retention, arguing that EMS is a distinct profession, not a subset of fire or a stepping stone to nursing.[33]
- **AAA (Ambulance Association):** Represents private ambulance companies. They lobby heavily for reimbursement reform (H.R. 3443) because their profit margins are decimated by the current Medicare rate structure, which often pays below the cost of service.[34]

Section IV: The "Nuclear" Threat — Fentanyl, WMDs, and the Department of War

The legislative and structural battles described above are taking place against a backdrop of radical geopolitical shifts in 2025. The reclassification of the drug war into a literal war has profound implications for EMS operational doctrine and liability.

4.1 The Designation of Fentanyl as a WMD

On December 15, 2025, President Donald Trump signed an Executive Order designating illicit fentanyl and its core precursor chemicals as Weapons of Mass Destruction (WMD).[35]

- **Rationale:** The order argues that fentanyl is "closer to a chemical weapon than a narcotic," citing its lethality in trace amounts (2mg) and its potential for weaponization

by terrorists.[35] The definition includes "core precursor chemicals" such as Piperidone, broadening the scope of interdiction.

- **Operational Shift:** This moves the fentanyl crisis from a Public Health Emergency (HHS/CDC domain) to a National Security Threat (DHS/DOD domain). For EMS, this changes the Rules of Engagement. An overdose call is no longer just a medical emergency; it is potentially a HazMat/WMD scene requiring higher levels of Personal Protective Equipment (PPE), decontamination protocols, and coordination with specialized threat assessment units.[35]

4.2 The "Department of War"

In a parallel move, the Department of Defense was renamed the **Department of War** (DOW) via Executive Order on September 5, 2025.[38] While largely symbolic in title, the rhetoric of "readiness and resolve" and "maximum lethality" signals a shift in domestic posture.

- **Secretary Pete Hegseth:** The new Secretary of War, Pete Hegseth, has emphasized a culture shift away from "tepid legality" toward "violent effect".[39] This aggressive stance has already manifested in reports of "kill everyone" orders and "double tap" strikes on drug boats in the Caribbean and Eastern Pacific, blurring the lines between military action and law enforcement.[40]
- **Militarization of EMS:** With the domestic drug war now framed as a WMD conflict, civilian EMS may find itself increasingly integrated into quasi-military operations. The NAEMSP and other medical direction bodies will face pressure to align protocols with TCCC (Tactical Combat Casualty Care) standards not just for SWAT, but for routine drug interdiction responses.
- **Funding Implications:** If fentanyl is a WMD, EMS agencies may be able to access Department of War or Homeland Security funding streams previously reserved for anti-terrorism, bypassing the stagnant DOT block grants. However, this comes with the condition of militarized training and compliance.[37]

4.3 The Legal Quagmire

The designation of fentanyl as a WMD raises immense legal questions for EMS. If a paramedic enters a "WMD scene" (a fentanyl lab or overdose site) without WMD-level protection, is the agency liable? Furthermore, the aggressive posture against "narco-terrorists" implies a battlefield operational environment. The Washington Post has reported on potential war crimes allegations involving the Secretary of War's directives, suggesting that domestic support operations could carry significant legal exposure for participating agencies.[41]

Section V: The Exodus and the Digital Enclosure

The structural failures (DOT), labor conflicts (Fire vs. Private), and escalation of risks (WMD) have culminated in a massive workforce exodus, complicated by a new layer of digital restrictions.

5.1 The Great Resignation and the Nurse Pipeline

The EMS workforce is hemorrhaging talent at an unsustainable rate.

- **Turnover Rates:** The 2024 Ambulance Employee Workforce Turnover Study found that annual turnover for EMTs and paramedics ranges from 20-30%, meaning an agency effectively replaces its entire workforce every 3-4 years.[42] In New York State, the number of active EMS responders declined by 17.5% between 2019 and 2022.[43]
- **The Wage Gap:** The primary driver is economic. In 2025, the median salary for a paramedic is approximately **$58,410**, while a Registered Nurse (RN) earns **$93,600**—a gap of over $35,000 annually.[44]
- **The "Bridge" Phenomenon:** EMS is increasingly viewed not as a career but as a stepping stone. Paramedic-to-RN "bridge" programs are proliferating, allowing medics to leverage their experience for expedited nursing degrees. Medics are leaving the streets for the hospital, where they earn significantly more for safer working conditions, better hours, and professional respect.[44]
- **Burnout:** 73% of EMS providers report experiencing burnout or compassion fatigue, driven by forced overtime, sleep deprivation, and the moral injury of a failing system.[43]

5.2 The Digital Enclosure: VPN Bans and App Store Laws

Just as the physical environment for EMS becomes more dangerous (WMDs), the digital environment is becoming more restrictive, impacting the ability of independent EMS to operate efficiently.

The VPN Bans (Wisconsin AB 105 / Michigan HB 4938):

Proposed legislation in Wisconsin and Michigan seeks to ban or heavily restrict the use of VPNs to prevent minors from bypassing age verification on adult sites.45

- **Technical Impact:** These bills would force ISPs to block "encrypted tunnels" or require websites to block traffic from known VPN IP ranges. For EMS, this is catastrophic. Modern EMS relies on **SASE (Secure Access Service Edge)** and corporate VPNs to transmit HIPAA-compliant patient data (EKGs, stroke scales, patient history) from the ambulance to the hospital.
- **The Telehealth Threat:** Rural EMS relies on telemedicine to connect paramedics with doctors for "Treatment in Place." If VPN technologies are criminalized or technically obstructed by state firewalls, the secure data pipeline required for these advanced consults collapses.[46]

The Texas App Store Accountability Act (SB 2420):

Effective January 1, 2026, this law requires strict age verification and parental consent for every app download by a minor, and imposes strict liability on "developers".48

- **Operational Friction:** While aimed at social media, the broad definition of "developer" and the requirement for "commercial reasonable methods" of age verification creates hurdles for EMS agencies using "Bring Your Own Device" (BYOD) policies. If a young EMT (under 18) needs to download a protocol app or scheduling tool, the friction of parental consent and verification creates administrative chaos.

- **The "Permissioned" Internet:** This law represents a shift toward a digital environment where anonymity is erased. For EMS, which deals with sensitive patient data, the forced collection of ID data by third-party app stores introduces new vector for data breaches.[50]

Section VI: Future Forecast — Best and Worst Case Scenarios

The convergence of the 1966 structural flaw, the 2025 militarization, and the workforce collapse points to a bifurcation of futures.

6.1 Worst Case Scenario: The Militarized Collapse

- **Fragmentation:** H.R. 3443 (MEDS Act) fails to pass due to budget hawkishness. Reimbursement remains tied to transport.
- **Ambulance Deserts:** Independent and private EMS agencies go bankrupt. Rural America becomes a vast "ambulance desert" where response times exceed 45 minutes.[31]
- **Total Fire Takeover:** In urban centers, Fire-based EMS (empowered by IAFF) absorbs all remaining third services (like MedStar). Costs skyrocket due to pension obligations, but clinical innovation stagnates under the "single role" civilian model which treats medics as second-class citizens within the fire house.
- **WMD Liability:** A fentanyl mass-casualty event occurs. EMS providers, lacking military-grade WMD training, refuse to enter the "hot zone" or suffer casualties. The "Department of War" deploys National Guard medical units to police domestic streets, effectively militarizing prehospital care permanently.
- **Digital Dark Age:** VPN bans in key states force EMS agencies to revert to radio and paper charting to avoid liability, slowing down care and increasing medical errors.

6.2 Best Case Scenario: The Healthcare Integration

- **Legislative Victory:** H.R. 3791 passes, finally counting EMS providers accurately. This data forces Congress to pass H.R. 3443, decoupling payment from transport.
- **The Federal Shift:** Recognizing the WMD threat, the Executive Branch moves the *medical* oversight of EMS from DOT to a new joint office between HHS and the Department of War (for threat defense). This unlocks healthcare dollars for "Treatment in Place."
- **Professionalization:** Wages rise to match nursing parity due to increased reimbursement. The "Para-EMT Act" successfully pipes veterans into the system, stabilizing the workforce.
- **Exemptions:** Legislators in TX, WI, and MI carve out broad exemptions for healthcare and public safety in digital ID/VPN laws, allowing secure EMS data networks to flourish.

6.3 Correlating the Forecast

The trajectory will likely be determined by the tension between the **"Department of War"** mindset (centralized, tactical, force-oriented) and the **"Digital Enclosure"** laws (state-level, restrictive, privacy-eroding).

- If the Federal Government (Department of War) asserts dominance, it may override state VPN bans for national security personnel (including EMS), creating a two-tier internet where first responders have access that civilians do not.
- The **Texas ID/App Store** laws serve as a harbinger of a "permission-based" internet. For EMS, this means the end of "bring your own device" (BYOD) policies. Agencies will be forced to issue government-controlled hardware to bypass age-verification/VPN blocks, further centralizing control and increasing costs.

Conclusion

The US EMS system is suffering from a congenital defect—its 1966 placement in the Department of Transportation—which has metastasized into a systemic failure. Treated as a shipping industry by the federal government, a revenue stream by fire departments, and a tactical unit by the new Department of War, EMS has lost its core identity as a healthcare practice. Without immediate legislative correction (H.R. 3443) and a unified federal home that prioritizes patient care over tire rotation, the system will not survive the twin pressures of the fentanyl WMD crisis and the great workforce exodus.

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Works cited

1. accidental death and disability - International Journal of Paramedicine, accessed December 17, 2025, https://internationaljournalofparamedicine.com/index.php/ijop/article/download/2521/2238/7342
2. Forty years ago, men and women of conviction, courage, and determination harnessed the national vision set forth in the White Pa - NWCEMSS, accessed December 17, 2025, https://www.nwcemss.org/assets/1/illinois_ems_education_committee/Testimony_for_IDPH_12-11.pdf
3. The White Paper and the Future: Where to Now? - HMP Global Learning Network, accessed December 17, 2025, https://www.hmpgloballearningnetwork.com/site/emsworld/article/12239745/the-white-paper-and-the-future-where-to-now
4. Why is EMS a part of the DOT? A look at The White Paper | Lucky Sci, accessed December 17, 2025, https://www.luckysci.com/2012/12/why-is-ems-a-part-of-the-dot-a-look-at-the-

white-paper/
5. Non-Emergency Utilization of EMS: Contributing Factors and Strategies to Promote Effective Care with Appropriate Resources - DigitalCommons@UNMC, accessed December 17, 2025, https://digitalcommons.unmc.edu/cgi/viewcontent.cgi?article=1127&context=coph_slce
6. Facilitating EMS Turnaround Intervals at Hospitals in the Face of Receiving Facility Overcrowding - NAEMSP, accessed December 17, 2025, https://naemsp.org/wp-content/uploads/2023/10/PEC-EMS-Turnaround.pdf
7. Chapter: 2 History and Current State of EMS, accessed December 17, 2025, https://www.nationalacademies.org/read/11629/chapter/4
8. A National Strategy to Promote Prehospital Evidence-Based Guideline Development, Implementation, and Evaluation, accessed December 17, 2025, https://prehospitalguidelines.org/wp-content/uploads/2018/01/National-Strategy-to-Promote-Prehospital-EBGs.pdf
9. Was the creation and legalization of firefighter paramedic programs inevitable despite parts of the medical society having objections to letting firefighters do medical stuff and lobbying against them? - Reddit, accessed December 17, 2025, https://www.reddit.com/r/ems/comments/1b0oxfl/was_the_creation_and_legalization_of_firefighter/
10. The New EMS Imperative: Demonstrating Value - Fitch and Associates, accessed December 17, 2025, https://fitchassoc.com/wp-content/uploads/2017/06/ICMA-InFocus-Report-The-New-EMS-Imperative.pdf
11. THE "80/20" ISSUE: AN UPDATE - Fire Engineering, accessed December 17, 2025, https://www.fireengineering.com/firefighting/the-80-20-issue-an-update/
12. Fort Worth EMS – Welcome to the City of Fort Worth, accessed December 17, 2025, https://www.fortworthtexas.gov/departments/fire/services/ems
13. Fort Worth Fire Department to take over EMS services from MedStar on July 1 - YouTube, accessed December 17, 2025, https://www.youtube.com/watch?v=UdFHo_B867Y
14. IAFF, Fort Worth, TX Local 440 support plan for fire-based EMS model, accessed December 17, 2025, https://www.iaff.org/news/iaff-fort-worth-tx-local-440-support-plan-for-fire-based-ems-model/
15. Proposed framework would ease EMS transition from MedStar to City | MedStar911, accessed December 17, 2025, https://www.medstar911.org/2025/03/02/proposed-framework-would-ease-ems-transition-from-medstar-to-city/
16. Should MedStar dispatchers have fire union representation? Fort Worth, union debate, accessed December 17, 2025, https://www.keranews.org/government/2024-10-02/should-medstar-dispatchers-have-fire-union-representation-fort-worth-union-debate
17. Texas city's EMS response times scrutinized months after fire department takeover - EMS1, accessed December 17, 2025,

https://www.ems1.com/fire-ems/texas-citys-ems-response-times-scrutinized-months-after-fire-department-takeover
18. Fort Worth to Launch Fire-Based EMS July 1: Saginaw Set to Join the Transition, accessed December 17, 2025, https://saginawtx.org/_T6_R197.php
19. Fort Worth Fire Department will soon take over EMS services as Medstar dissolves - WFAA, accessed December 17, 2025, https://www.wfaa.com/article/news/local/fort-worth-fire-department-will-take-over-ems-services-as-medstar-dissolves/287-df748cf2-2a21-4285-a2e2-b6607d8aaa89
20. What are the differences between public and private EMS and ambulance services? - EMS1, accessed December 17, 2025, https://www.ems1.com/private-public-dispute/articles/private-vs-public-ambulance-services-whats-the-difference-WTgJNJgR4KlljlV9/
21. Santa Clara County Emergency Medical Services Agency Financial Analysis, accessed December 17, 2025, https://files.santaclaracounty.gov/exjcpb1541/migrated/SCC%20EMS%20%20Emergency%20Ambulance%20System%20Fin%20Analysis%20Report-Sep%202022.pdf?VersionId=FEALeTu40y7H35tDuBPxp3lPOybZTU6p
22. US HB3791 | 2025-2026 | 119th Congress | LegiScan, accessed December 17, 2025, https://legiscan.com/US/bill/HB3791/2025
23. Text - H.R.3791 - 119th Congress (2025-2026): EMS Counts Act | Congress.gov, accessed December 17, 2025, https://www.congress.gov/bill/119th-congress/house-bill/3791/text
24. 'EMS Counts Act' introduced in Congress - EMS1, accessed December 17, 2025, https://www.ems1.com/legislation-funding/articles/ems-counts-act-introduced-in-congress-R4C0U3MRR4iyTPEa/
25. Bill would add cross-trained EMTs, paramedics to federal classification system - EMS1, accessed December 17, 2025, https://www.ems1.com/labor-issues/bill-would-add-cross-trained-emts-paramedics-to-federal-classification-system
26. Cosponsors - H.R.3791 - 119th Congress (2025-2026): EMS Counts Act, accessed December 17, 2025, https://www.congress.gov/bill/119th-congress/house-bill/3791/cosponsors
27. H. R. 3443 - Congress.gov, accessed December 17, 2025, https://www.congress.gov/119/bills/hr3443/BILLS-119hr3443ih.pdf
28. Bill tracking in US - HR 3443 (119 legislative session) - FastDemocracy, accessed December 17, 2025, https://fastdemocracy.com/bill-search/us/119/bills/USB00095399/
29. H.R. 3443 - Modernizing EMS Delivery and Sustainability (MEDS) Bill - NAEMSP, accessed December 17, 2025, https://naemsp.org/news/h-r-3443-modernizing-ems-delivery-and-sustainability-meds-bill/
30. Text - H.R.2220 - 119th Congress (2025-2026): PARA–EMT Act of 2025 | Congress.gov, accessed December 17, 2025, https://www.congress.gov/bill/119th-congress/house-bill/2220/text

31. Rural EMS - National Rural Health Association, accessed December 17, 2025, https://www.ruralhealth.us/nationalruralhealth/media/documents/advocacy/2025/rural-ems-7-15-25.pdf
32. LD-2 Disclosure Form, accessed December 17, 2025, https://lda.senate.gov/filings/public/filing/e9b98c33-b1ee-4c8a-8ed0-57873aa1f1ee/print/
33. Advancing EMS For Our Workforce and Communities - NAEMT's, accessed December 17, 2025, https://www.naemt.org/docs/default-source/2017-publication-docs/naemt-annual-report-2022-04-25-2023.pdf?sfvrsn=40ddf493_4
34. Online Legislative Service - NAEMT's, accessed December 17, 2025, https://www.naemt.org/advocacy/online-legislative-service
35. DESIGNATING FENTANYL AS A WEAPON OF MASS ..., accessed December 17, 2025, https://www.whitehouse.gov/presidential-actions/2025/12/designating-fentanyl-as-a-weapon-of-mass-destruction/
36. Fact Sheet: President Donald J. Trump Designates Fentanyl as a Weapon of Mass Destruction - The White House, accessed December 17, 2025, https://www.whitehouse.gov/fact-sheets/2025/12/fact-sheet-president-donald-j-trump-designates-fentanyl-as-a-weapon-of-mass-destruction/
37. Congresswoman Lauren Boebert Reintroduces Legislation to Protect our Children by Classifying Fentanyl as a Weapon of Mass Destruction, accessed December 17, 2025, https://boebert.house.gov/FentanylWMDAct
38. Fact Sheet: President Donald J. Trump Restores the United States Department of War, accessed December 17, 2025, https://www.whitehouse.gov/fact-sheets/2025/09/fact-sheet-president-donald-j-trump-restores-the-united-states-department-of-war/
39. Trump Renames DOD to Department of War, accessed December 17, 2025, https://www.war.gov/News/News-Stories/Article/Article/4295826/trump-renames-dod-to-department-of-war/
40. The Fill from the Hill: Department of War and potential crimes - University Times, accessed December 17, 2025, https://csulauniversitytimes.com/the-fill-from-the-hill-department-of-war-and-potential-crimes/
41. Littwin: Defense Secretary Pete Hegseth, an unserious man, is accused of serious war crimes. Or worse. - The Colorado Sun, accessed December 17, 2025, https://coloradosun.com/2025/12/03/hegseth-trump-crimes-opinion-littwin/
42. Please Support EMS Workforce Development - naemt, accessed December 17, 2025, https://naemt.org/docs/default-source/emshd-2025/emshd-2025-workforce---para-emt-act.pdf?sfvrsn=3010ff93_2
43. New York data supports sounding the alarm on the EMS workforce crisis | NYSenate.gov, accessed December 17, 2025, https://www.nysenate.gov/newsroom/in-the-news/2025/shelley-b-mayer/new-york-data-supports-sounding-alarm-ems-workforce

44. Registered Nurse Vs. Paramedic: What's The Difference? | NurseJournal.org, accessed December 17, 2025, https://nursejournal.org/resources/rn-vs-paramedic/
45. They're Coming for Your VPN: Michigan's Insane Anti-Cloud Law, accessed December 17, 2025, https://www.youtube.com/watch?v=ljPV_vO6fvs
46. Michigan's Shocking VPN Ban Explained, accessed December 17, 2025, https://www.youtube.com/watch?v=q9j-T9mQmiw
47. Proposed Age Verification Bills and the Impact on VPNs and SASE: What You Need to Know, accessed December 17, 2025, https://www.todyl.com/blog/proposed-age-verification-bills-and-the-impact-on-vpns-and-sase
48. Texas' Expanding Digital Compliance Frontier: Taking Aim at Your Websites & Your Personal Identity - KJK, accessed December 17, 2025, https://kjk.com/2025/11/05/texas-expanding-digital-compliance-frontier/
49. App Store Age Verification Laws: Your Questions, Answered. | Privacy World, accessed December 17, 2025, https://www.privacyworld.blog/2025/10/app-store-age-verification-laws-your-questions-answered/
50. Davis Wright Tremaine LLP Files First Amendment Challenge to Texas App Store Law, accessed December 17, 2025, https://www.dwt.com/about/news/2025/10/dwt-files-1a-challenge-to-texas-app-store-law